contact G.E.H.A at (800) 821-6136. Authorization of a fertility medication does not imply authorization of any related fertility procedures.

Assisted Reproductive Technologies (ART) (High Option)

Assisted Reproductive Technologies (ART) are considered medically necessary for the following conditions:

A. Unexplained infertility
B. Diminished ovarian reserve
C. Tubal factor infertility
D. Male factor infertility
E. Endometriosis
F. Ovulatory dysfunction
    1. When ovulation induction has not resulted in conception
    2. Poor response to ovulation induction
    3. Hyper-response to ovulation induction where there is a risk for ovarian hyperstimulation or a multiple gestation
G. Failure to achieve conception after 3 unsuccessful attempts with IVI, IUI or ICI treatment.
H. Iatrogenic Infertility (High Option)

When Assisted Reproductive Technologies (ART) are not covered:

A. When using autologous oocytes in the setting of a very poor or futile prognosis; OR
B. When there is a failure to respond to ovarian stimulation (e.g., as demonstrated by failure to achieve at least 1 follicle >12 mm in diameter); OR
C. ART cycle does not demonstrate the attainment of at least 1 embryo suitable for transfer; OR
D. Lack of viable spermatozoa; OR
E. Ovarian failure where a couple is attempting conception with their own gametes; OR
F. Recurrent aneuploidy or ≥5 unexplained losses; OR
G. Greater than 2 consecutive ART cycles without adequate egg quality or production, fertilization and/or embryo quality or development

Intracytoplasmic Sperm Injection (ICSI) (High Option)

Intracytoplasmic Sperm Injection (ICSI) ICSI is considered medically necessary for the following:

A. Male factor infertility
    1. “Male factor” infertility is an alteration in sperm concentration and/or motility and/or morphology in at least two sperm analyses, collected 4 weeks apart.
B. After failed conventional insemination